Royal Devon University Healthcare NHS Foundation Trust

Telephone Triage Standard Operating Procedure
Eastern and Northern Services

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Note: This document has been assessed for any equality, diversity or human rights implications			

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Full History **New SOP for development of Triage service**		***//*** **Status: Draft** or **Final** DRAFT	
Version	Date	Author (Title not name)	Reason
1.0	28.12.23	Transformation Midwife	New document, to create a clinical pathway for the development of a Telephone Triage service for the Maternity Unit.
1.1	June 2024	Transformation Midwife	Additional bullet points to section 7.7: • An OASI / perineum with a breakdown and any signs of systemic infection (pyrexia, fatigue, spreading erythema, nausea and vomiting, loss of appetite, tachycardia, hypotension). See Clinical Guideline for repair of perineal trauma for further information. • Perineal concerns/breakdown, if no signs or symptoms of a systemic infection can see GP or CM/W. A referral must be made to the Perinatal Pelvic Health Midwife for any women with a broken-down perineum.

Associated Trust Policies/ Procedural documents:	Maternity Triage Guideline Latent phase of labour guideline
Key Words:	Triage, Telephone, Maternity, Fetal, Liquor, Bleeding, reduced fetal movements, Hypertension, PET, PIH, IPC, Obstetric Cholestasis, USS, Presentation USS, CTG, SROM, PROM, PPROM, postnatal, Wound, Perineum
In consultation with and date: Obstetric team; Risk manager; Senior management team; Midwives.	
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KEY POINTS OF THIS PROCEDURAL DOCUMENT:

The Trust operates a dedicated Maternity Telephone Line for all Maternity Triage phone calls and contacts from Northern and Eastern service users.

Maternity Telephone Triage is a 24-hour service.

1. INTRODUCTION

1.1 Childbearing women use the telephone to access maternity services at times of uncertainty. Midwives use information provided by women to determine their need for a face to face assessment. When managing telephone triage, it is vital that midwives have strong communication skills, experience and sound clinical knowledge (Bailey et al, 2019). The development of a consistent approach to telephone triage assists in mitigating risk (Finlay and Brown, 2013).

1.2 Within this document each time the word woman/women is used we are referring to the term women or birthing people.

2. PURPOSE

2.1 This SOP provides an overview of the standards for telephone triage to ensure care is safe and appropriate care is provided by following a standardised assessment criteria.

2.2 The SOP provides clear roles and responsibilities for all maternity staff and students for telephone triage, in line with national professional standards and recommendations.

3. DEFINITIONS

- **SOP** – Standard Operating Procedure
- **CDS** – Central Delivery Suite
- **DAU** – Day Assessment Unit
- **FMAU** – Fetal maternal assessment unit
- **LWC** – Labour Ward Co-ordinator
- **CTG** – Cardio Tocograph
- **USS** – Ultrasound Scan
- **OC** – Obstetric Cholestasis
- **PET** – Pre-eclampsia
- **PIH** – Pregnancy Induced Hypertension
- **IOL** – Induction of Labour
- **SROM** – Spontaneous Rupture of Membranes
- **PROM** – Prolonged Rupture of Membranes
- **PPROM** – Premature Prolonged Rupture of Membranes
- **RFM** – Reduced Fetal movements
- **CM/W** – community midwife
- **FTSC** – first trimester screening
- **RDUH –** Royal Devon University Healthcare NHS Foundation Trust
- **APNW –** Ante/postnatal Ward (Eastern)
- **CS –** Caesarean Section

4. DUTIES AND RESPONSIBILITIES OF STAFF

4.1 The role of the Matron

- Triage matron to produce a staffing roster to cover both the Triage and the Telephone Triage Service
- Maintaining the service through up to date SOP and Guidelines where appropriate
- Managing performance, sickness and development of staff

4.2 The role of the Midwife

The midwife working on telephone triage will;

- Be responsible for triaging callers and documenting telephone calls in EPIC using appropriate telephone triage risk assessment flow sheet and then ordering an admission to the appropriate unit; Appendix 1
- Ensure they are in the triage context for EPIC when receiving phone calls.
- Referral of Woman to the appropriate unit and department. Eastern maternity triage, Centre for women's health, 24/7 service and Northern Triage, Ladywell Unit Basset ward 24/7 service.
- Liaising with triage staff and labour ward co-ordinators in the event of escalation.
- Provide personalised clinical advice within their scope of practice according to the NMC.
- Update labour ward Coordinator East and North

5. MAIN BODY OF THE STANDARD OPERATING PROCEDURE

5.1 Staffing

- Long Day Shift 07.30-20.30 1 midwife (if short shifts are allocated number of staff for cover stays the same but is split into 2 shifts.)
- Night Shift 20.00 – 07:30 1 midwife.
- Telephone triage staff should be protected from being re allocated to other areas at times of high acuity and must be staffed at all times.

5.2 Source of Referrals

- Antenatal referrals should be for those Woman >12 weeks and following FTSC for Eastern services and >18/40 for Northern services.
- Postnatal referrals should be up to 6-weeks post-delivery.

Referrals to the Maternity Triage via telephone triage may be from:

- Self - referral
- Community Midwife
- GP
- Emergency Department
- ANC, DAU or FMAU
- Ultrasound department
- Paramedics

5.3 Telephone Advice

- All telephone calls for clinical advice to Maternity Triage will be recorded on EPIC telephone triage flow sheets. See appendix 1
- Clinical advice must be given by a registered professional (or a midwifery student working under the direct supervision of a registered professional).
- The telephone triage assessment should be fully completed and will require the midwife to use the presenting history given alongside their clinical judgment to determine any appropriate advice.

Woman with current pregnancy complications or underlying medical history should have a lower threshold for advising attendance.

Clinical advice should also take into account:

- Unit booked with.
- Distance from the Maternity Unit and availability of transport.
- Time of Day.
- Any social complexities such as Safeguarding / Language Barriers.
- Number of contacts with the Maternity Triage Service.

6. INDICATIONS FOR REFERRAL

6.1 RDUH operates a dedicated telephone line for maternity triage telephone calls relating to these conditions but this is not an exhaustive list:

- Spontaneous rupture of membranes
- Early or established labour
- Abdominal pain
- Pre- term spontaneous rupture of membranes
- Antepartum bleeding
- Reduced/absent/sudden alteration in fetal movements
- Raised blood pressure
- Vaginal bleeding in pregnancy
- Postnatal readmissions up to 6 weeks
- Hyperemesis / dehydration
- Headache
- Fall / RTA
- Itching

6.2 Women with the following clinical presentations should always be invited in for assessment

- abdominal pain ***AND*** vaginal bleeding.
- reduced / absent / increased fetal movements.
- Any presence of a communication barrier whereby the clinician is not assured of the clinical history or maternal / fetal wellbeing provided by a telephone assessment.

6.3 Those who contact the service with an emergency clinical presentation such as:

- Severe abdominal pain
- Severe vaginal bleeding
- Imminent birth without the presence of a midwife or medical assistance

In these circumstances the caller should be advised to contact the emergency ambulance service and will require admission directly to labour ward or CDS.

The labour ward coordinator / CDS coordinator for the appropriate site should be advised of all impending emergency admissions to ensure the appropriate obstetric, midwifery, neonatal and theatre teams are on standby.

7.0 ADMISSION CRITERIA

- Maternity triage will provide an assessment service for all pregnant Woman greater than 12/40 following FTSC in Eastern services and greater than 18/40 weeks gestation (Northern services) requiring unplanned Obstetric or Midwifery Care who have booked at North Devon District Hospital or Royal Devon Exeter Hospital or for anyone who is currently staying in the area.
- All women who call the triage service for a third time in relation to the same concern or within a 24-hour period should be invited in for a review
- **This is not an exhaustive list and each person should be assessed on an individual basis, using the EPIC telephone triage system and clinical expertise. If uncertain, seek more senior advice**.

ALWAYS LISTEN TO THE CALLER

7.1 Suspected labour

Advise to attend if any <u>one</u> of the following:

- Suspected labour < 37/40
- Term and contracting regularly and strong:
  - Multip e.g. 2-3 contractions in 10 mins, lasting 40 seconds
  - Primip e.g. 3-4 contractions in 10 minutes lasting 60 seconds
- 3rd call to triage with ?labour
- Has tried analgesia and other coping mechanisms which are not effective
- Previous quick labour (less than 2 hours from onset of contractions/SROM)
- Any concerns about obstetric history/complications (e.g. preterm, booked CS)
- Wants to come into unit

Face to face review not required/telephone advice only:

- Blood-stained mucus at term
- Appears to be in early labour (see advice below)

Advise to call back if:

- Contractions are every 5 minutes and lasting 40-60 seconds
- Membranes rupture (especially if brown, green, pink/red)
- PV bleeding
- Baby's movement pattern changes

Advice for latent phase of labour:

- Eat nutritious, high-energy foods
- Drink fluids to thirst (not excessively and try to monitor amount)
- Regular bladder emptying (4hrly at least and monitor)
- Rest – sleep/relax between contractions when able to do so
- If unable to rest then mobilise (birthing ball, walking, upright positions)
- Massage from birth partner may be helpful
- Use TENS machine if available
- Utilise bath if available
- Breathing techniques/ hypnobirthing
- Use paracetamol as per instructions, caution if SROM as may mask signs of infection
- Reassure that it is normal for labour to take some time to establish, especially with the 1st baby.

Advice for imminent birth at home.

- Stay on the phone with the caller, providing reassurance and advice on preparation for birth.
- The callers partner/support to call 999 and arrange immediate ambulance to attend urgently. When requesting an ambulance in this circumstance, 'Obstetric Emergency' needs to be said to call handler to instigate a Category 1 ambulance. This is due to a person birthing with no medical assistance. If there is no support at home, telephone triage midwife to get support from unit and call 999.
- If planned homebirth community midwife to also be asked to attend.

7.2 Antenatal abdominal pain

Always ask the nature of the pain:

- Describe the type of pain (Sharp, dull ache, tightness? If multiparous does it feel like labour pain?)
- Where is the pain? (Upper abdomen, pelvis, central, all over?)
- Duration? (Does it last for a few seconds or minutes? Is it constant?)
- Frequency? Has this pain occurred before, is it a regular pain, does it come every 5 minutes? 30 minutes?)
- Do they have any UTI symptoms or recent UTI infection?
- Do they have a history of pelvic girdle pain? If so, has this changed e.g. different severity or location of pain?

Advise to attend if:

- Any abdominal pain <37/40
- Moderate or severe constant pain
- Inability to void
- Backache with other concerning symptoms

Face to face review not required/telephone advice only:

- Chronic or mild pain on mobilising only
- Generalised Backache

To call back if:

- Pain/contractions increase
- Pain is accompanied with PV bleeding
- Change in fetal movements

Advise:

- Continue to monitor
- Try some paracetamol
- Try a warm bath

7.3 Antenatal bleeding

Always ask the nature of the bleeding to decide urgency of attendance:

- When did it start?
- Has there been any injury/bump to abdomen?
- Is the bleeding post- coital?
- Are they Rh negative?
- Can they describe the colour? (Fresh red, pink discharge, brown old loss, mucus)
- Can they quantify it? (Spots on wiping, full pad, continuous trickle)
- Is the loss ongoing or has it settled?

To attend if:

- Any PV bleeding at any gestation that is not mixed with mucus
- Blood-stained mucus show at < 37/40

Face to face review not required/telephone advice only:

- Blood-stained mucus at term

Call back if:

- Any pain or contractions
- Have further PV bleeding
- Change in any fetal movements

Advice:

- Keep any pads that have PV loss and bring to hospital when attending
- Put on a fresh pad
- <12/40 bleeding. If significant bleeding to attend ED, If not significant 111/GP or CM/W
- <18/40 bleeding. If significant bleeding to attend ED, if not significant 111/GP or CM/W

7.4 Fetal movements

Take history:

- How have the movements changed? (Less often, not as strong, increased, change in the usual pattern?)
- When is the last time they felt the movements were normal?
- Assess for risk factors of SGA/stillbirth
- Is this the 1st episode of RFM

<24/40

- If NO fetal movements felt by 24/40 order a scan to assess fetal growth, liquor volume and Doppler flow.

>22/40 - <28/40

- Woman to contact their community team/midwife to be assessed the same day by a community midwife if not able to contact or not heard back within 2 hours to attend triage.
- If out of hours to attend triage.

≥28/40

- TCI immediately for assessment + CTG monitoring

7.5 Spontaneous rupture of membranes (SROM)

Take history:

- What time did the ?SROM occur?
- Was it a gush of fluid or is it damp underwear?
- What colour is the loss?
- Colour should be documented as clear, blood stained or meconium
- Is it continuing to leak?
- Are they wearing a pad?
- Are they known/previous GBS+?

To attend if:

- Convincing history of SROM at any gestation to attend within 1-2hrs
- Known or suspected SROM with meconium or blood-stained liquor, offensive liquor, high temperature or other signs of infection – immediate attendance
- Known GBS and suspected SROM

Face to face review not required/telephone advice only:

- Uncertain history of SROM (No GBS known) - advise to wear a pad and call back with an update in 1-2 hours for further discussion and plan.

Call back if:

- They think the membranes have gone or pad shows liquor not urine
- If pain/contractions increase
- PV bleed
- Change in fetal movements

7.6 Headache

Take history of:

- When did the headache start?
- Where is the headache? (Frontal, behind eyes, all over?)
- Do they suffer with migraines pre-pregnancy? Is this episode a change in symptoms?
- Is it associated with any visual disturbances or other symptoms of PET?
- Are they known PET in this pregnancy or previous pregnancy?
- How have they tried to relieve the headache already?
- Is it a hot day? Could they be dehydrated?
- If postnatal, did they have an epidural? Could it be a Dural tap? Is the headache better when laid down/ in a dark room

To attend if:

- Moderate or severe headache (not migraine) and /or visual disturbances, epigastric pain, fit/loss of consciousness

Face to face review not required/telephone advice only:

- Migraine sufferer and no difference to normal symptoms.

Call back if:

- Headache gets worse, persists or develop any other symptoms e.g. visual disturbance
- Change in fetal movements
- PV loss

Advice:

- Take paracetamol, rest, eat and increase fluid intake (not excessively)
- If any neurological symptoms such as numbness/weakness or Thunderclap headache to attend ED

7.7 Postnatal Mother

To attend if:

- Heavy/continuous lochia after 5 days
- Offensive smelling at any time or if passing large clots (ask to save any clots/take pictures if possible send via EPIC patient message. Appendix 1.1)
- Suspected mastitis/infection/temperature (>37.7, if feels hot feverish or extremely cold/unwell)
- An OASI / perineum with a breakdown and any signs of systemic infection (pyrexia, fatigue, spreading erythema, nausea and vomiting, loss of appetite, tachycardia, hypotension). See Clinical Guideline for repair of perineal trauma for further information.

Face to face review not required/telephone advice only:

- Increased lochia after being active, sleeping, breastfeeding and has now settled again
- Any concerns with a baby that is considered to have feeding difficulties but otherwise appears clinically well, following neonatal triage call proforma then consider: 1.) Can community maternity team or Tiverton maternity unit provide feeding support. 2.) If not, then refer to APNW Eastern or Basset Ward North. Informing the appropriate ward with the plan for admission for feeding support. 3.) If baby has previously fed well and/or there are concerns that the baby is unwell following the neonatal telephone proforma, the baby must be referred to PAU
- LSCS wound concerns but no signs or symptoms of infection (ask to take a picture and send via EPIC patient messenger.) GP or CM/W to review if non-urgent.
- Perineal concerns/breakdown, if no signs or symptoms of a systemic infection can see GP or CM/W. A referral must be made to the Perinatal Pelvic Health Midwife for any women with a broken-down perineum.

Call back if:

- Lochia becomes heavy and continuous or offensive
- Sudden onset of abdominal pain or starts to feel unwell

Advice:

- If minor to contact community midwife
- If any neurological symptoms such as numbness or weakness, then advise to attend ED

7.8 Postnatal Baby

To call 999 immediately if:

- Any indication of life-threatening condition
- Any concerns regarding baby's breathing, colour or tone
- If baby is unresponsive

To attend PAU (East) and ED (North) immediately if:

- Concerns with baby's condition which are not deemed life threatening
- Rash
- Green vomit
- If baby has previously fed well and/or there are concerns that the baby is unwell following the neonatal telephone proforma, the baby must be referred to PAU
- If the caller is still unsure and un reassured following neonatal assessment, refer to PAU

Advice:

- Feeding concerns with a well baby – support to be given via phone as required. For ongoing support and advice, refer to community midwife and infant feeding team
- Sticky or offensive cord – advice cleaning of area and review by community Midwife. To see GP if continual signs of infection
- Refer to infant feeding team and community midwives during day time.

Call back if:

- Any further concerns or for further advice.

7.9 Unwell/other

To attend triage if:

- ?UTI, pain/stinging when passing urine or passing urine more frequently at any gestation
- Temperature >37.7 or if feels hot, feverish or extremely cold, any obvious infection site e.g. abdominal wound, perineum, breasts or Covid 19
- Persistent vomiting – unable to tolerate any diet/fluids. Ensure discussion regarding any other symptoms including abdominal pain.

To attend FMAU/DAU if:

- Persistent itching of hands/feet (Order appointment on EPIC for DAU/FMAU)

Face to face review not required/telephone advice only:

- Diarrhoea/vomiting or hyperemesis if able to keep small amounts of water down and passing urine (use clinical skills to assess if needs to be seen e.g. other risk factors/symptoms, not typical hyperemesis, any changes to vomiting).
- Mild to moderate mental health concerns (check if well supported at home and refer to specialist/email community midwife and IAPT services).

Call back if:

- Continues to feel unwell, pain/contractions, PV loss or RFM

Advice:

- Take some paracetamol (if not allergic), rest and see if it resolves
- Mild- moderate mental health – The Community Perinatal Team offer same day response consultation to professionals Monday-Friday (10:00-16:00). You can telephone the team for advice on making a referral or the urgency of a referral. Exeter 01392 674964 Torbay 01803 396590 North Devon 01271 443177 West Devon 01822 813070
- If referrer concerned that person is at risk of hospital admission then refer to Mother and Baby Unit Outreach Team (their referral criteria is also 2 working days but they can also provide advice on same day) – https://www.dpt.nhs.uk/ourservices/pregnant-women-andnew-mothers/mother-and-babyunit/mbu-outreach-service
- Moderate/severe mental health – Devon first response service 24/7 0801968708 or call 999

Woman out of area to attend closest ED department

7.10 Hyperemesis

- <14/40 calling with hyperemesis, please complete PUQE assessment appendix 1.2 if at risk of dehydration advise to come in to Wynard (EAST) and page SHO or gynae SHO and inform them of imminent arrival to unit
- <18/40 Northern advise ED, 111, GP CM/W as appropriate.

7.11 Covid-19 related concerns

If the woman is feeling unwell, could it be Covid?

- New continuous cough
- Any shortness of breath
- High temperature
- Diarrhoea or vomiting
- Change in taste or smell

Be aware of the increased risks in Black Asian and Mixed ethnicity people and those from the most deprived communities and have a lower threshold for review and admission.

If attending triage then ask for the woman to wait in the car/carpark when she arrives, call triage to let us know she is here, wear a face mask at all times and we will collect her from outside and take her to a designated room. Liaise with labour ward coordinator.

Always advise women and families to follow the latest national guidance

Complete a VTE assessment, if requiring Fragmin to order prescription from GP, advise fluids, mobilising and to monitor fetal movements.

7.12 Other

For other miscellaneous issues consider where the best place is to refer to e.g. GP, walk in centre, ED. some examples include:

- If >6 weeks PN – see GP/ ED
- If <12/40 East or <18/40 North to contact GP/ED/111
- Any non-pregnancy issue (sprained/broken limbs, insect bites)
- Mental health issues/concerns that need assessment (significant change in mood/behaviour or confusion)
- Chest pain
- Breathing difficulties
- Blinding headache ('thunderclap') if not a migraine sufferer and have no pregnancy related issues
- Tender swollen, red, hot to touch calf
- Any loss of consciousness or if an epileptic experiencing more fits than normal
- Sudden weakness/numbness especially on one side of body, trouble speaking/seeing or lack of co-ordination

ALWAYS LISTEN TO THE WOMAN

8. DID NOT ATTEND

- If the service user does not attend in the expected time frame, the triage midwife must make attempts to contact the person back to understand the reason for DNA and document.
- If the caller has a transport issue and the recommendation is for the person to attend, discuss alternative options including friends, family and local taxi service. If there appears to be no alternative and there are financial concerns, liaise with labour ward coordinator (appropriate site) and site management.

- If the caller is not wishing to attend due to hospital anxieties, carefully consider the reasons for attendance. If the person requires a health care professional review, consider the use of CMW or GP.
- However, if the recommended advice is attend triage, then this must be reiterated and documented. Ask how attending could be made easier and discuss these supportive adjustments with the labour ward coordinator. Review and follow any emotional wellbeing plans already in place.
- If there is a change in condition, re-risk assess the previous call alongside the current update. This may require an escalation in attendance time and consider the need for contacting the ambulance service on behalf of the caller (for example – precipitate labour/ongoing heavy PV bleeding). In this case if possible, stay on the phone to support and document, whilst initiating 24hr reception to contact Ambulance Control.
- If they are reporting the symptoms have improved, carefully re-risk assess the previous call alongside the current update. Any episode of reduced movements, PV bleeding, abdominal pain should still be investigated and recommended admission
- If unsure about anything, always seek further advice from either a senior midwife or consultant/senior registrar and document who you have discussed this with.

9. ARCHIVING ARRANGEMENTS

The original of this SOP, will remain with Triage leads. An electronic copy will be maintained on the Trust intranet, SOP.

Archived electronic copies will be stored on the Trust's “archived policies” shared drive, and will be held indefinitely. A paper copy (where one exists) will be retained for 25 years.

10. PROCESS FOR MONITORING COMPLIANCE AND EFFECTIVENESS OF THE SOP

To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
Audit – EPIC telephone proforma completed and note. Calls answered time Calls waiting Calls abandoned	AUDIT	Audit Midwife/ Triage telephone lead. To Maternity Governance monthly

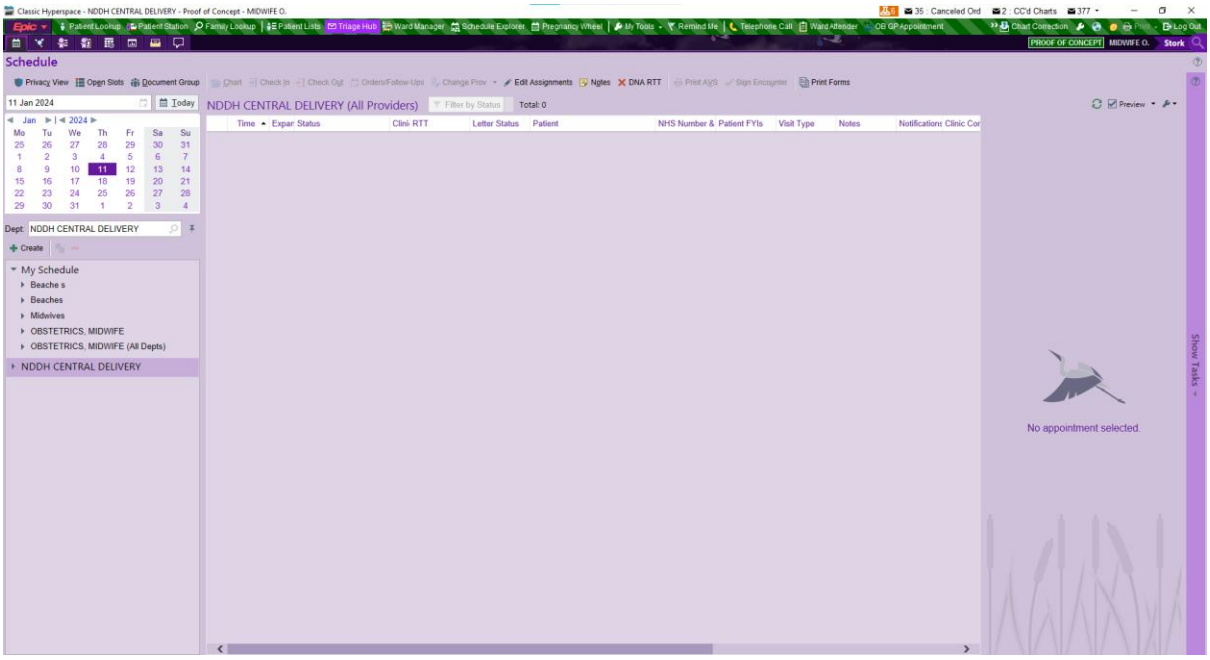
11. REFERENCES

Bailey, C. M., Newton, J. M. and Hall, H. G. (2019) ‘Telephone triage in midwifery practice: A cross-sectional study’, International Journal of Nursing Studies (91). Available at: http://website60s.com/upload/files/telephone-triage-in-midwifery-practice-a-cr_2019_international-journal-of-n.pdf (Accessed 20 August 2021).

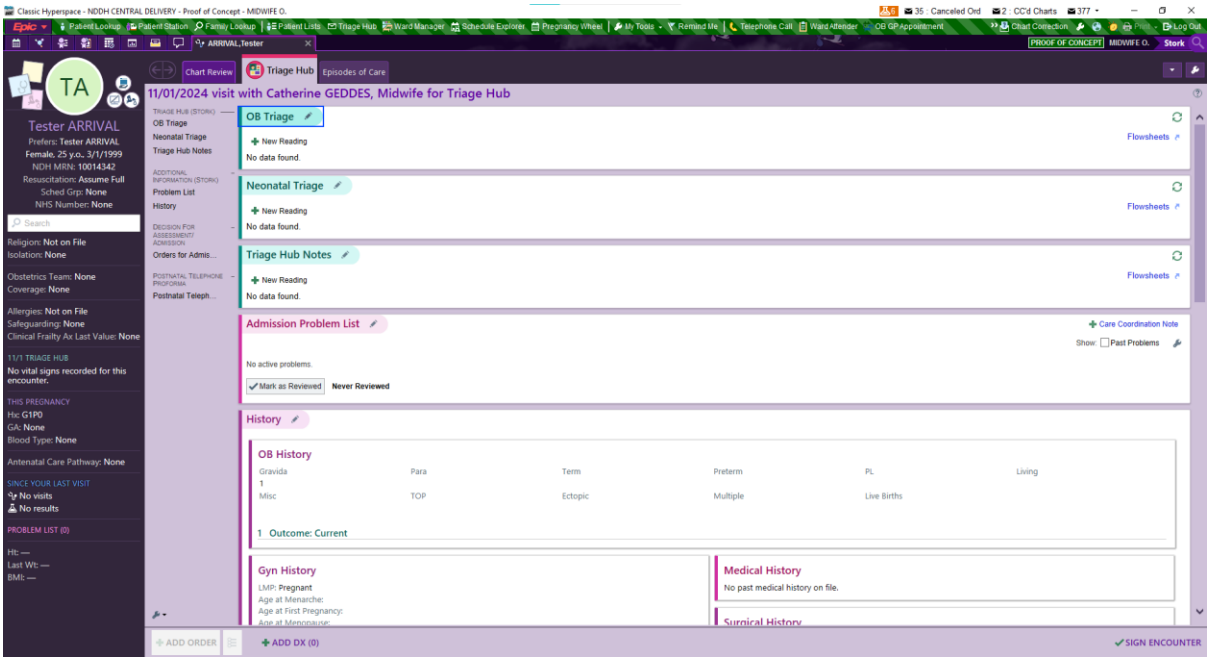
Finlay, D. and Brown, S. (2013) ‘Maternity telehealth: ringing the changes’, Practicing Midwife, 16 (11), pp. 32-34.

APPENDIX 1: EPIC

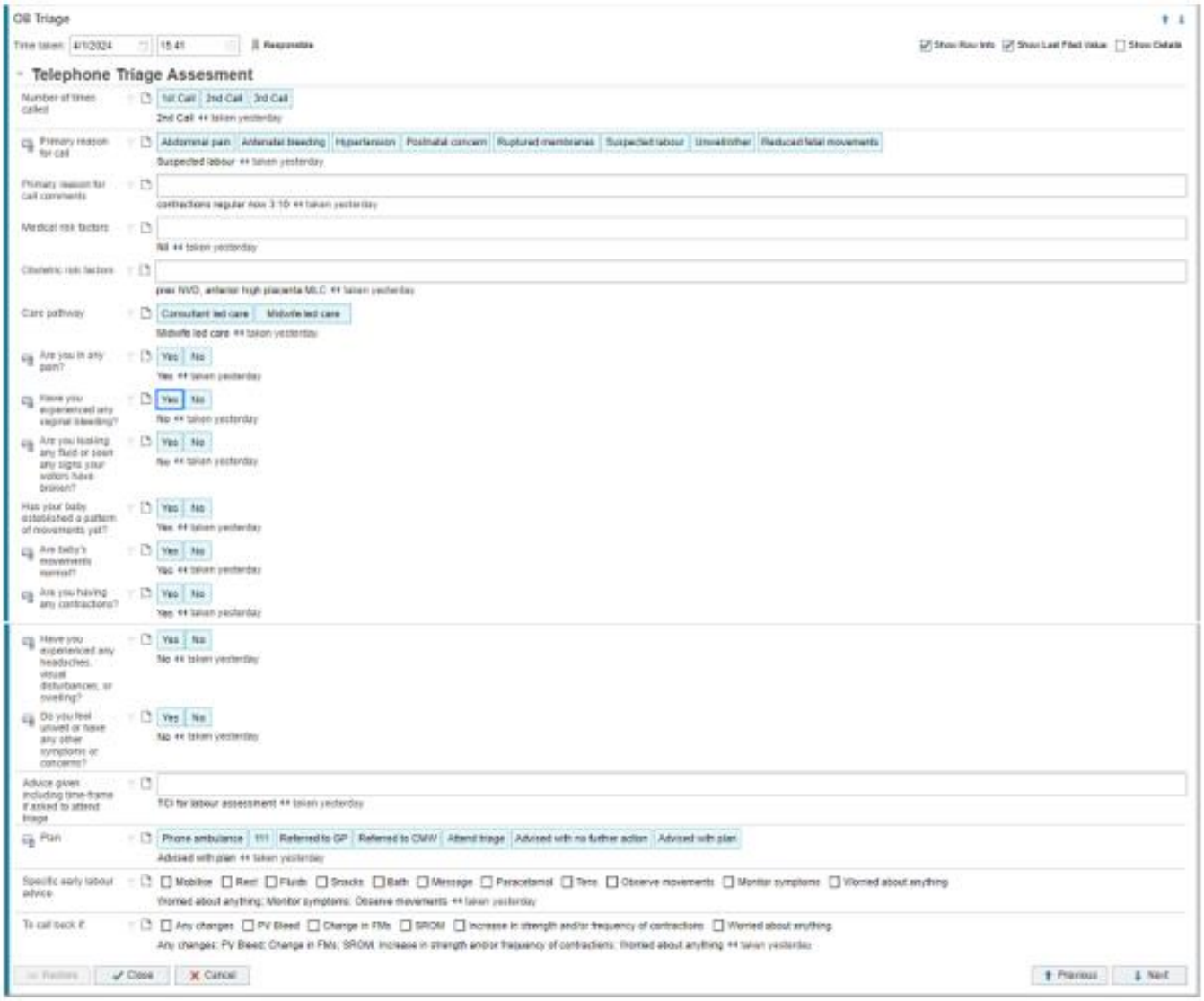
1) **Select Triage hub from top drop-down bar.**



2) **Select the appropriate encounter and click the pencil.**



3) Complete Telephone assessment flow chart for appropriate call (Triage, Postnatal or neonatal.)



Neonatal Triage

Time taken: 4/1/2024 15:13 Responsible

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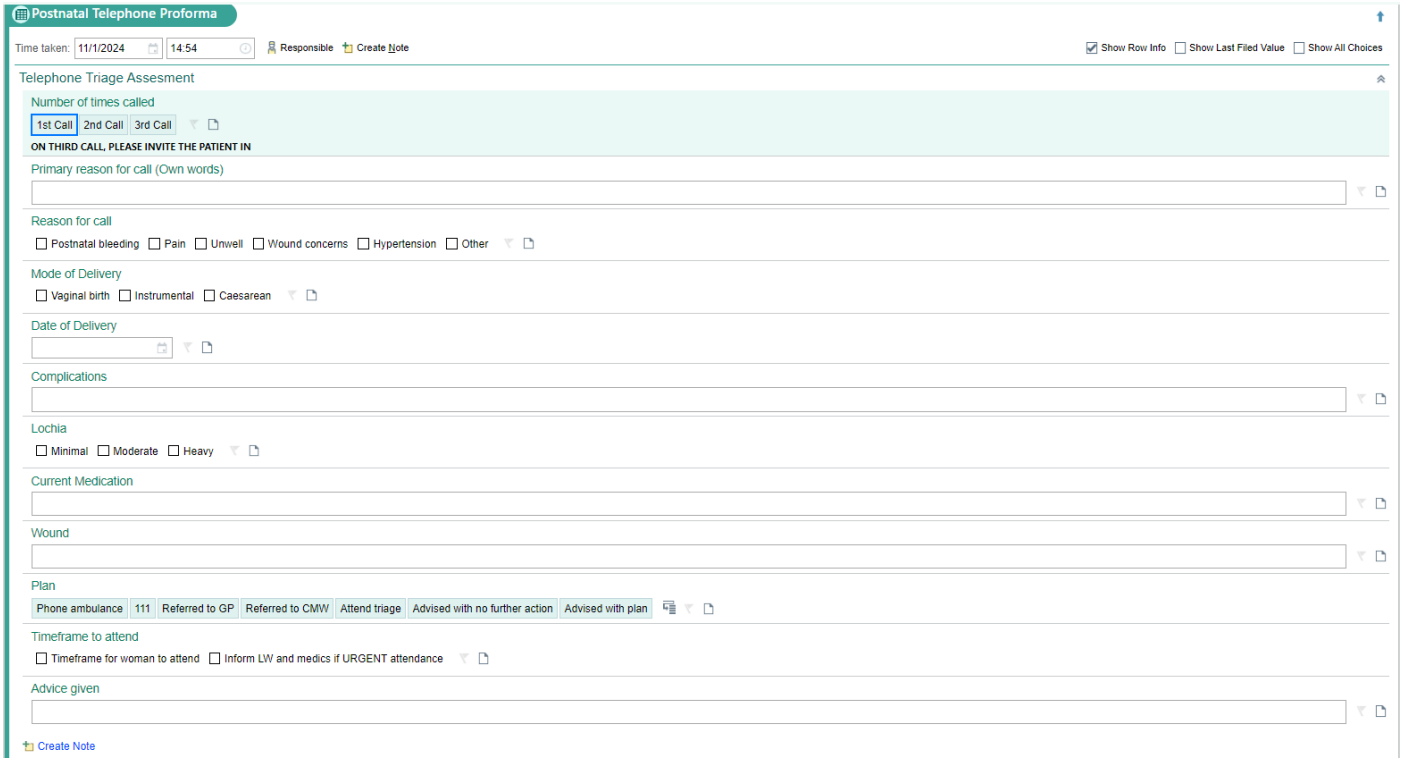
Neonatal Telephone Triage Proforma

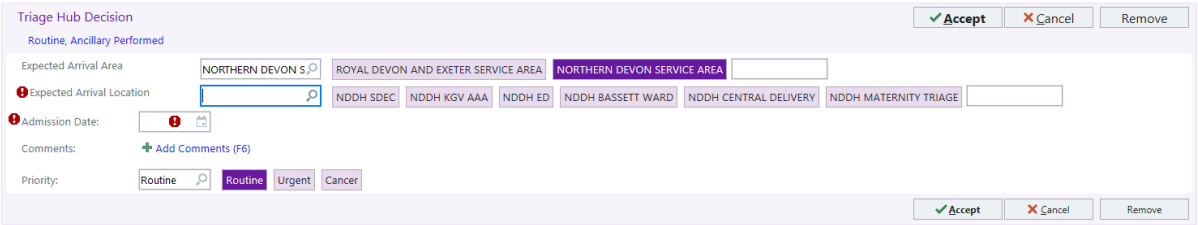
Number of times called	1st Call / 2nd Call / 3rd Call
Responding department/service	Maternity Inpatient / Maternity Community / Neonatal / Other
Primary reason for call	Feeding concern / Infant crying / Unwell infant / Jaundice / Injury / Rash / Vomiting / Other
Reason for call in parent's own words	
Relevant history	
Is your baby breathing normally? Is your baby making any noises when breathing?	Normal breathing / Abnormal breathing e.g. grunting
When you handle your baby, is the tone	Normal tone / Abnormal tone
Do you have any worries about your baby's behaviour? Is your baby drowsy, lethargic or difficult to wake? Is your baby jittery, irritable or crying more than normal?	Normal behaviour / Abnormal behaviour
When you look at your baby's skin, has the colour changed? Is your baby more yellow, grey, blue or pale than before?	Normal skin colour / Abnormal skin colour
Does your baby's temperature feel normal? When you place your hand on baby's back does it feel cold or too hot, sweaty or clammy?	Normal temperature / Abnormal temperature
Do you have a thermometer? Have you taken your baby's temperature?	Yes / No
Has your baby got a new rash, bruising or any marks on their skin?	Yes / No
Do your baby's eyes look normal? Are the whites of the eyes discoloured? Is there any redness or stickiness around either eye?	Normal / Abnormal
Does your baby's mouth look normal? Is your baby's mouth pink and moist? Are there any white patches or coating in the baby's mouth?	Normal / Abnormal
Cord	On / Off
Does your baby's umbilicus look normal? Is there any fresh bleeding? Is the skin around the base of the cord red, swollen or inflamed?	Umbilicus normal / Umbilicus abnormal
How is your baby feeding?	Breastfeeding / Formula / Mix
Do you have concerns about your baby's feeding? Has your baby stopped feeding? Are you worried your baby is not feeding enough?	Concerns / No concerns
How many times has your baby passed urine in the last 24 hours?	
How many times has your baby passed stool in the last 24 hours?	
Has your baby been vomiting?	Yes / No
Where is your baby sleeping?	
Do you have any other concerns not mentioned?	Yes / No
Advice given	Phone ambulance immediately / 111 / Attend A&E / Speak to GP / Community midwife / Health visitor / Referred to APNW / Referred to PAU / Referred to Bramble / Advised with no further action / Other
Plan of care	
To call back if	☐ New concerns or symptoms arise, or needing further advice ☐ Worsening of current symptoms ☐ Unable to contact / No contact from recommended services
Staff designation	Midwife / Nurse / Maternity / Neonatal support worker / HCA / Doctor / Other

Restore | ✔ Close | ✘ Cancel | ↑ Previous | ↓ Next



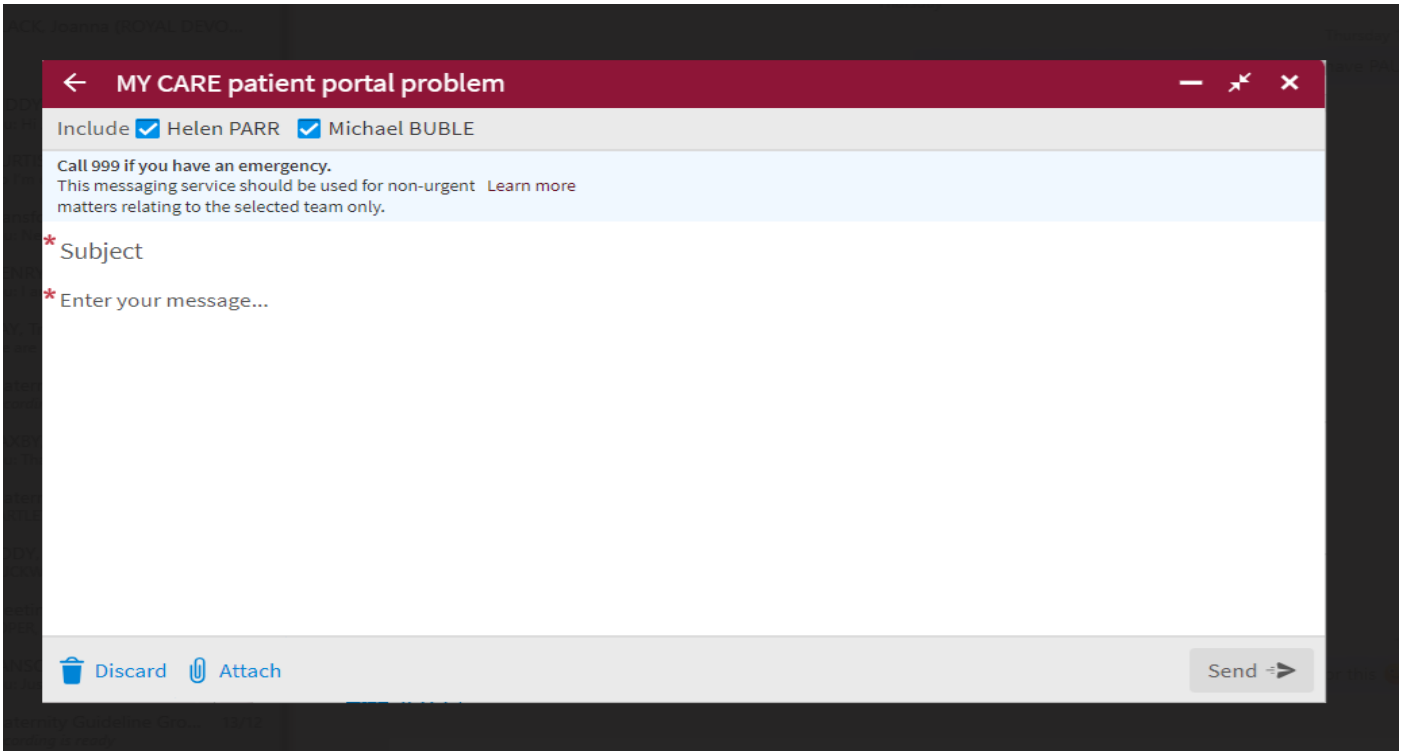
4) Place the order for admission

5) Choose the correct expected arrival location, date and sign encounter

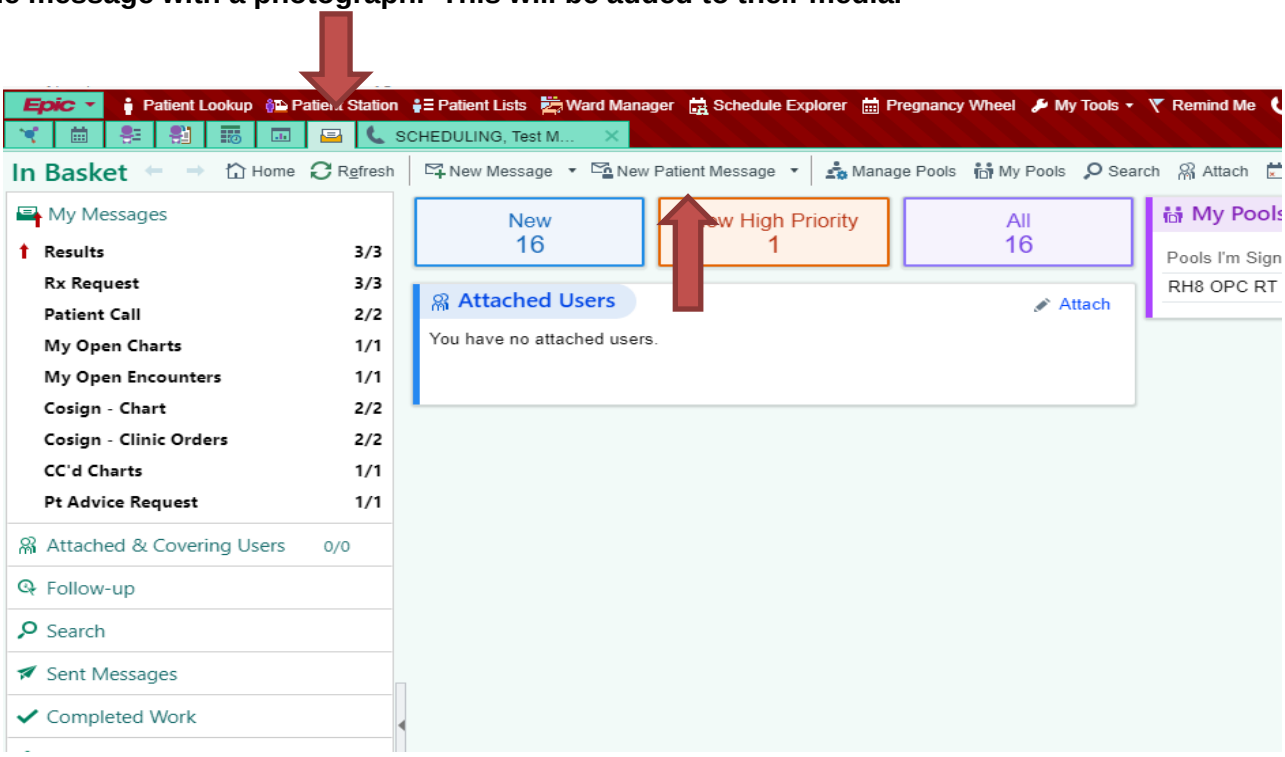


APPENDIX 1.1: EPIC SENDING PICTURE

1] Patients can send a picture from their phones on Mycare using this proforma.



2] If the patient is unable to send to the telephone midwife, the midwife can message them through EPIC via a new patient message, select the appropriate patient using MRN and ask them to reply to the message with a photograph. This will be added to their media.



APPENDIX 1.2: PUQE ASSESSMENT TOOL and EPIC

PUQE score chart for hyperemsis

In the last 12 hours for long long have your felt nauseated or sick to your stomach?					
Not at all (1)	1 hr or less (2)	2-3hr (3)	4-6hr (4)	More than 6hr (5)	
In the last 12 hours how many times have you vomited or thrown up?					
7 or more times (5)	5-6 times (4)	3-4 times (3)	1-2 times (2)	I did not throw up (1)	
In the last 12 hours how many times have you had retching or dry heaves without bringing anything up?					
7 or more times (5)	5-6 times (4)	3-4 times (3)	1-2 times (2)	None (1)	

Mild ≤ 6; Moderate = 7-12; Severe = 13-15.

PUQE Telephone Assessment

1: Go to the More Menu in your telephone call.

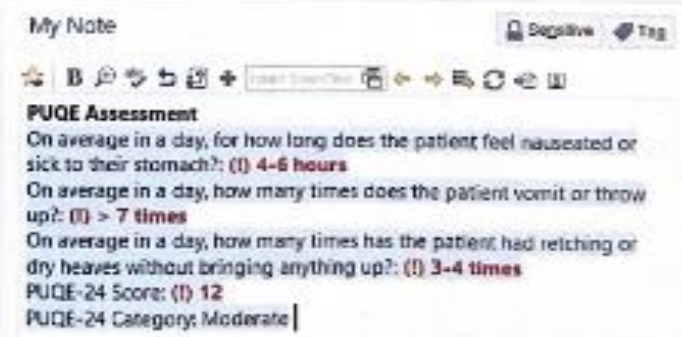
2: Open Flowsheets

3: Find the search box for flowsheet templates (top right). Search PUQE, you will find it the first time under the facility list but can then add it to your preference list.

4: Complete the PUQE assessment

5: When writing your note you can pull the assessment in by doing .PUQE

This can be used within the other triage smart phrases



APPENDIX 2: COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the guideline/SOP	Midwives and obstetricians.
The key changes if a revised document	New document no revision
The key objectives	The purpose of the SOP is to provide guidance for an initial outpatient telephone call and to provide an initial assessment of maternal and fetal wellbeing and to direct the caller using the approportionate care pathways within Maternity services.
How new staff will be made aware of the procedure/guideline and manager action	PEF midwives during new staff induction process Orientation process Emails BSOTS training
Specific Issues to be raised with staff	Exclusion criteria and the need for prompt escalation when clinical picture changes should be emphasised
Training available to staff	Triage team leads
Any other requirements	Service users to have contact number and communication around this.
Issues following Equality Impact Assessment (if any)	No negative impacts
Location of hard / electronic copy of the document etc.	The original of this SOP, will remain with the author. An electronic copy will be maintained on the Trust intranet, SOP. Archived electronic copies will be stored on the Trust's “archived policies” shared drive, and will be held indefinitely. A paper copy (where one exists) will be retained for 25 years.

The Impact Assessment Form should be listed last in your appendices. It is not a requirement for Divisional/ departmental documents; however, it is encouraged where possible.

APPENDIX 3: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	Maternity Triage Telephone SOP
Division/Directorate and service area	Maternity Service
Name, job title and contact details of person completing the assessment	Joanna Black Transformation midwife
Date completed:	28.12.23

The purpose of this tool is to:

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. What is the main purpose of this document?

Clinical guidance

2. Who does it mainly affect? *(Please insert an "x" as appropriate:)*

Carers ☐ Staff ☒ Patients ☒ Other (please specify)

3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below? (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for Woman than for men)

Please insert an "x" in the appropriate box *(x)*

Protected characteristic	Relevant	Not relevant
Age	☐	☐
Disability	☐	☐
Sex - including*: Transgender, and Pregnancy / Maternity*	☐x	☐
Race	☐	☐
Religion / belief	☐	☐
Sexual orientation – *including: Marriage / Civil Partnership*	☐	☐

4. Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to… (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

none identified

5. Do you think the document meets our human rights obligations? ☒

Feel free to expand on any human rights considerations in question 6 below.

A quick guide to human rights:
• ***Fairness –*** *how have you made sure it treats everyone justly?* • ***Respect –*** *how have you made sure it respects everyone as a person?* • ***Equality –*** *how does it give everyone an equal chance to get whatever it is offering?* • ***Dignity –*** *have you made sure it treats everyone with dignity?* • ***Autonomy –*** *Does it enable people to make decisions for themselves?*

6. Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?

New document Patient and staff involvement will be included in the audit

7. If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact please note this below and how this will be monitored/addressed.

"Protected characteristic":	SEX - Maternity Service
Issue:	None
How is this going to be monitored/ addressed in the future:	NA
Group that will be responsible for ensuring this carried out:	NA